

Multimodal RAG for Med-VQA

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Introduction

- Why Medical VQA?
 - Med-VQA = answering clinical questions about medical images (X-ray, CT, MRI) with natural language.
 - Harder than general VQA: subtle anatomical features, pathology reasoning, and modality understanding.
 - Potential use cases:
 - Support radiologists and clinicians
 - Clinical training & education
 - Patient-facing explanations



ImageCLEF / LifeCLEF - Multimedia Retrieval in CLEF

Original Paper: Multimodal Retrieval-Augmented Med-VQA

- Multimodal Retrieval-Augmented Generation with LLMs for Medical VQA (MasonNLP @ MEDIQA-WV 2025).
 - Key idea:
 - Use a **general-domain instruction-tuned LLM**
 - Add a **lightweight RAG layer** that retrieves similar text + image exemplars from the dataset at inference time
 - Benefits:
 - Better grounding, fewer hallucinations
 - Improved schema adherence and clinical relevance

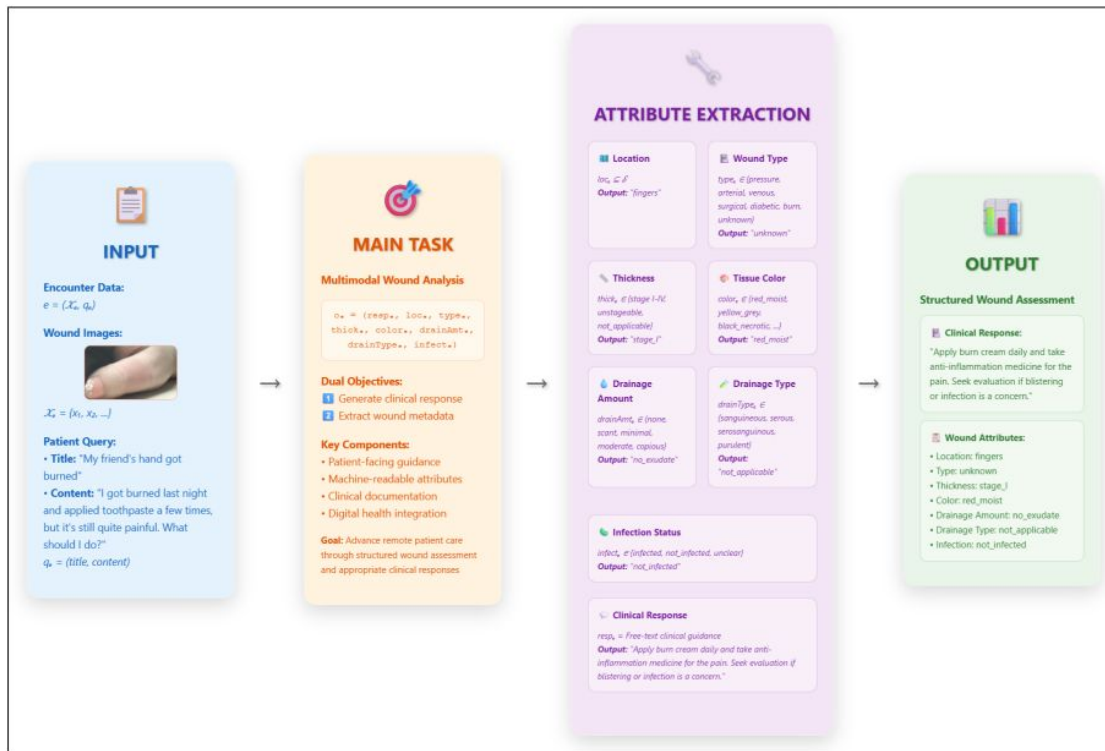
Project Goals



- Dataset: VQA-RAD – radiology visual QA benchmark with 451 test samples in our setting.
- Goals:
 - Reproduce a **baseline model** on VQA-RAD
 - Design a **Stage 2 improved model** inspired by retrieval-based and semantic ideas
 - Perform **systematic error analysis**:
 - Which question types fail most?
 - How does Stage 2 change the error distribution?

Baseline Model & Evaluation Setup

- Baseline pipeline:
 - Encode image
 - Encode question text
 - Fuse features → answer predictor
- Evaluation:
 - Strict exact-match accuracy on test answers
 - No credit for synonyms or more specific wording
- Raw baseline accuracy: 35.03%
- After counting clearly equivalent answers: 36.81%

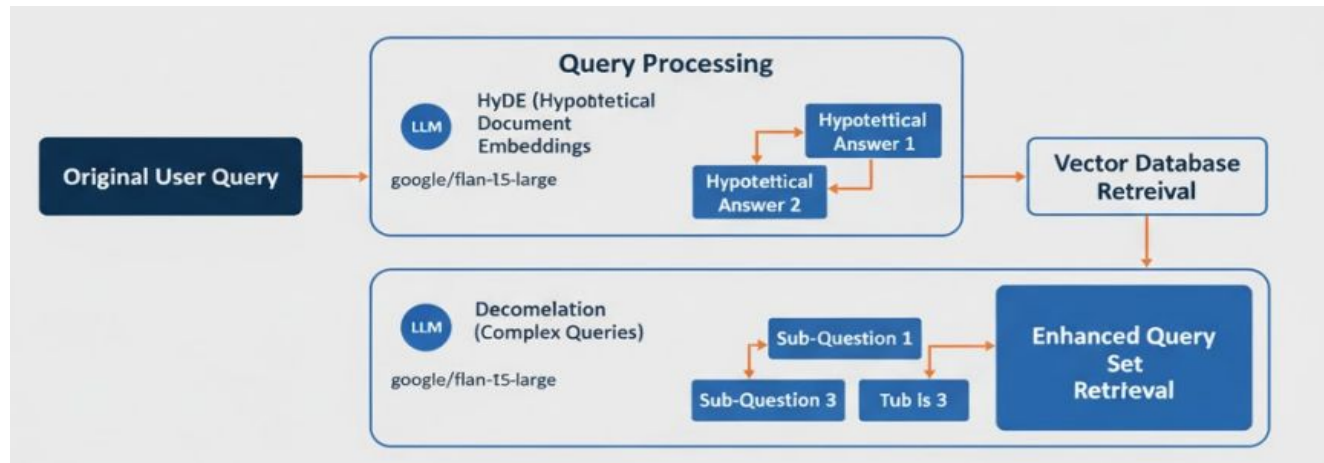


Performance Metrics

Error Category	Count	Percentage
Binary Errors	18	36%
Pathology ID Errors	12	24%
Localization Errors	9	18%
Anatomical Errors	6	12%
Modality/Sequence Errors	3	6%
Quantification Errors	2	4%

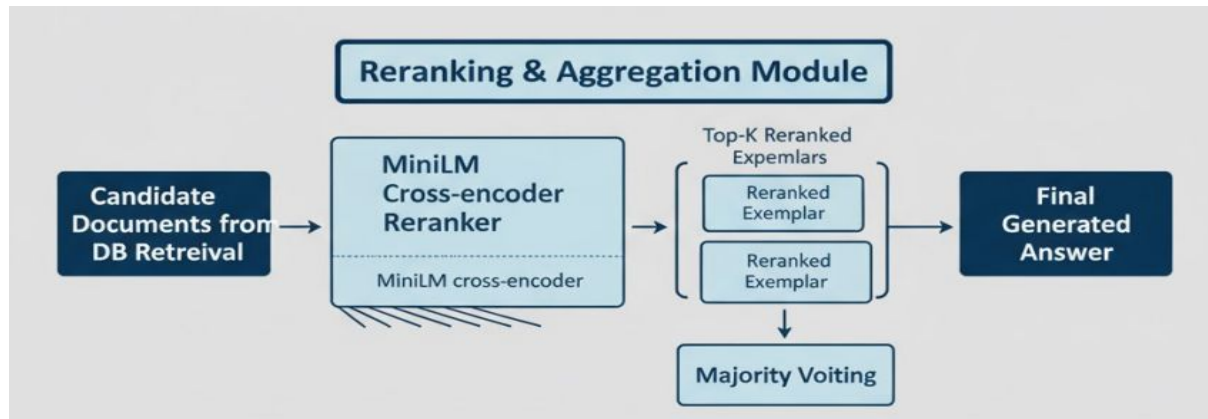
Hybrid Model: Query Rewriting

- Problem in MasonNLP
 - Retrieval only used the **raw question text**
 - Complex or multi-part questions often pulled **weak exemplars**
- Purpose:
 - Use LLM-driven (google/flan-t5-large) **query rewriting** to create richer variants before retrieval:
 - **HyDERewrite**: generate 1–2 hypothetical answers to turn into additional queries
 - **DecompositionRewrite**: split a complex question into up to 3 simpler sub-questions
 - More relevant exemplars for tricky cases (binary, pathology, localization)



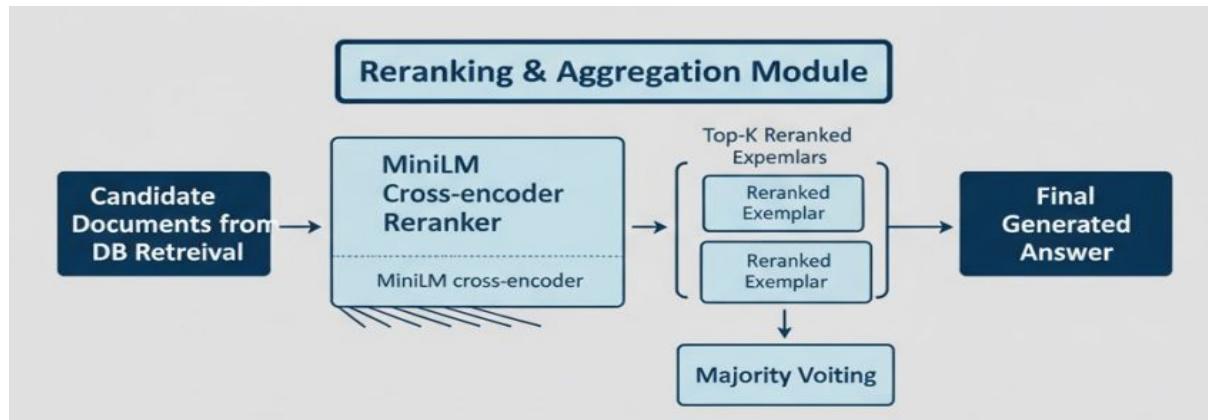
Hybrid Model: Reranker

- Problem in MasonNLP-style baseline
 - Baseline ranked candidates only by **embedding similarity**
 - Top hits were sometimes **lexically close but clinically off**, leading to noisy or misleading answers
- Purpose:
 - Add a **MiniLM cross-encoder reranker** to re-score:
 - How well each exemplar actually answers the question
 - Filters noisy exemplars; keeps the most clinically aligned ones



Hybrid Model: Majority Voting

- Problem in MasonNLP-style baseline
 - Baseline often copied **one exemplar's answer**
 - If that one exemplar was an outlier, the final prediction was wrong, even when other good exemplars existed
- Purpose:
 - Use **majority voting** across top-k exemplars to stabilize predictions and reduce the impact of any one bad exemplar
 - Smooths out outliers and **reduces variance** from noisy exemplars



Improvement: Baseline vs Stage 2

Accuracy Comparison

Metric	Baseline	Stage 2 Hybrid	Improvement
Overall Accuracy	36.81%	37.03%	+0.22%

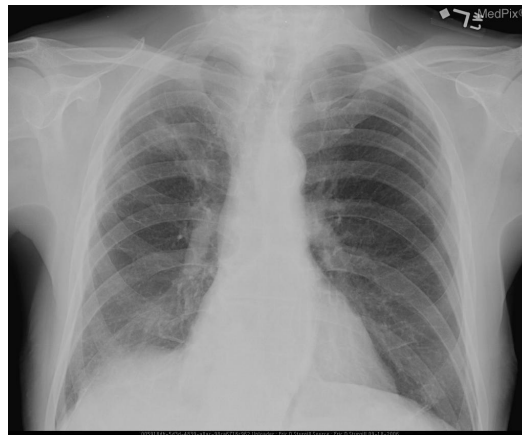
Error Rate Comparison

Error Type	Baseline	Stage 2 Hybrid	Improvement
Binary Classification Errors	36%	32%	-4%
Pathology Identification Errors	24%	20%	-4%
Localization Errors	18%	16%	-2%
Anatomical Structure Errors	12%	14%	+2%
Modality/Sequence Errors	6%	10%	+4%
Quantification Errors	4%	4%	0%

Qualitative Improvements

Case 1: Cardiac Assessment

- test_00129:
 - Question: "is the heart enlarged?"
 - Gold Answer: "no"
 - Baseline Prediction: "yes" ❌
 - Stage 2 Prediction: "no" ✅



Case 2: Mediastinal Evaluation

- test_00268:
 - Question: "is there a mediastinal shift?"
 - Gold Answer: "no"
 - Baseline Prediction: "yes" ❌
 - Stage 2 Prediction: "no" ✅



Results on Other Datasets - PathVQA

- Dataset: 5,000+ pathology images and 33,000+ Q–A pairs; more diverse and harder than VQA-RAD.
- Settings:
 - **Baseline:** no retrieval, no rewriting, no reranking (FLAN-T5 only).
 - **Hybrid RAG:** HyDE + decomposition + MiniLM cross-encoder reranker (text + image retrieval).
- Accuracy on PathVQA (test, n = 3000):
 - Baseline (no RAG): **26.7%**
 - Hybrid RAG: **32.4%**
 - **+5.7 percentage points** improvement

Results on Other Datasets - SLAKE

- Dataset: Semantically-labeled, bilingual Med-VQA dataset with ontology-style annotations and diverse question types.
- Settings:
 - **Baseline:** no retrieval, no rewriting, no reranking (FLAN-T5 only).
 - **Hybrid RAG:** HyDE + decomposition + MiniLM cross-encoder reranker (text + image retrieval).
- Accuracy on SLAKE:
 - Baseline (no RAG): **48.5%**
 - Hybrid RAG: **43.6%**
 - **-4.9 percentage points** drop in accuracy

Future Work & Takeaways

Future Work:

- Dataset-aware RAG: Tune or disable rewriting / reranking per dataset (e.g., SLAKE may need lighter RAG than VQA-RAD / PathVQA).
- Stronger components: Try larger or domain-tuned LLMs for HyDE / decomposition and LLM-based rerankers; improve vision encoders for retrieval.
- Smarter evaluation: Add semantic / ontology-based matching and small human audits so clinically valid answers aren't punished by string match.

Key Takeaways:

- A config-driven hybrid pipeline (query rewriting + reranker + majority voting) improves retrieval without changing the core VQA model.
- It helps on noisy, free-text datasets (VQA-RAD, PathVQA) but can hurt on structured SLAKE, so RAG must be tuned, not blindly applied.
- The framework is easy to extend—future teams can swap models or rewrite chains by just editing configs.

Thank you!